Royal Devon University Healthcare NHS Foundation Trust

Non-Medical Authorisation of Blood Components for Transfusion by Registered Practitioners Policy	
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Division/ Department responsible for Procedural Document	Specialist Services
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Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

Monitoring Information		**Strategic Directions – Key Milestones**	
Patient Experience	✓	Maintain Operational Service Delivery	✓
Assurance Framework	✓	Integrated Community Pathways	✓
Monitor/Finance/Performance		Develop Acute services	
CQC Fundamental Standards - Regulation**:**		Infection Control	
Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

Controlled document
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Version	Date	Author	Reason
1.0	05/07/2023	Transfusion Practitioner	Aligning policies and practice across both the Northern and Eastern Sites
1.0	03/08/2023	Transfusion Practitioner	Ratified by PBMG

Associated Trust Policies/ Procedural documents:	Transfusion Policy
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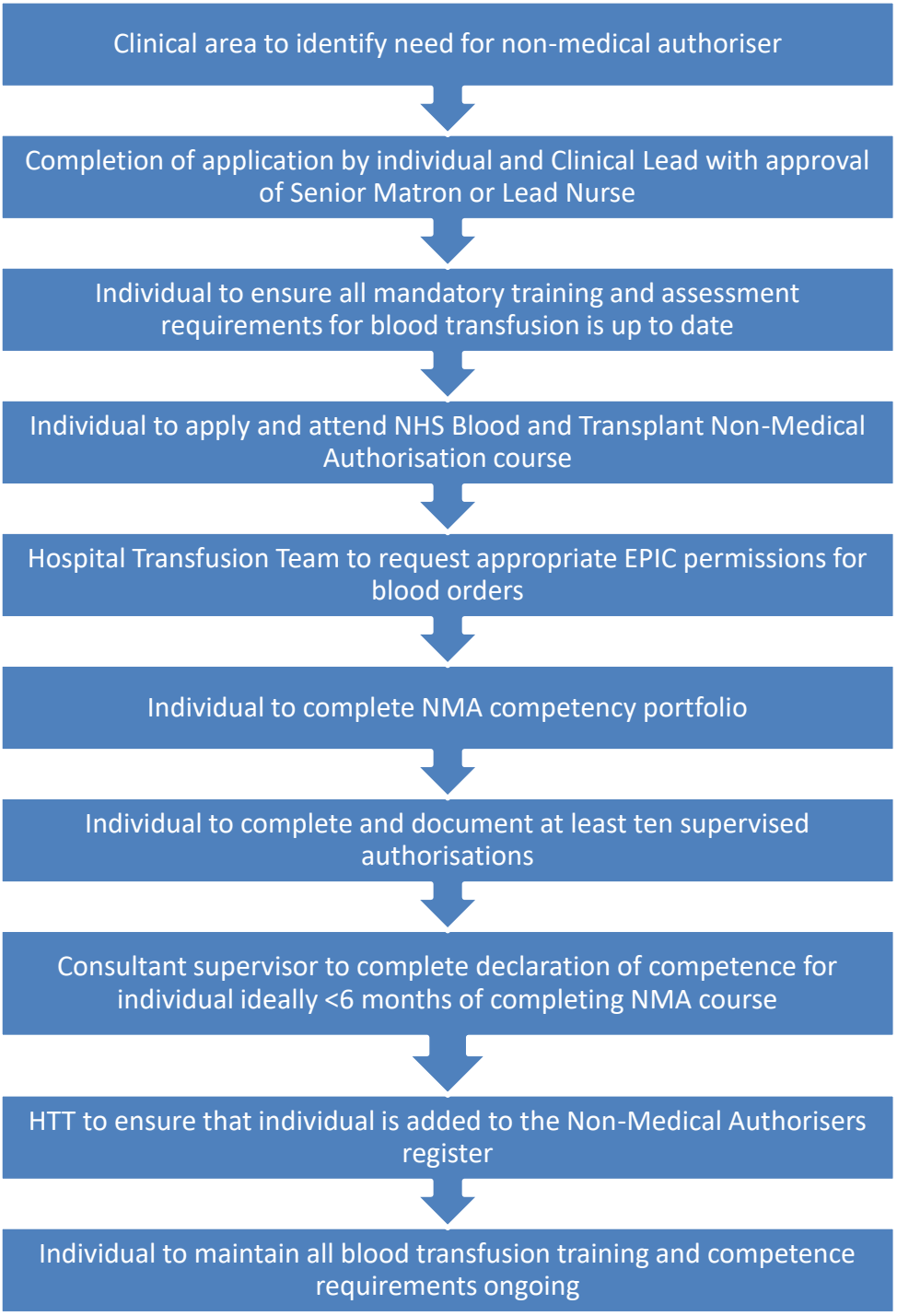
In consultation with and date: Hospital Transfusion Team Patient Blood Management Group	
Contact for Review:	Transfusion Practitioner
Executive Lead Signature: *(Applicable only to Trust Strategies & Policies)*	**To be added by Policies Administrator when uploading to Intranet**

CONTENTS

1 KEY POINTS OF THIS POLICY: 4
1 INTRODUCTION 5
2 PURPOSE 5
3 DEFINITIONS 5
4 DUTIES AND RESPONSIBILITIES OF STAFF 5
5 PROCESS 7
6 ARCHIVING ARRANGEMENTS 7
8 REFERENCES 8
9 BIBLIOGRAPHY 9
APPENDIX 1_APPLICATION FORM FOR NON-MEDICAL AUTHORISATION OF BLOOD COMPONENT TRANSFUSION 10
APPENDIX 2 PORTFOLIO OF EVIDENCE 11
APPENDIX 3_LOG OF SUPERVISED AUTHORISATION OF BLOOD COMPONENTS 16
APPENDIX 4_DECLARATION OF COMPETENCE FORM 18
APPENDIX 5_NON-MEDICAL AUTHORISING AUDIT TOOL 19
APPENDIX 6: COMMUNICATION PLAN 20
APPENDIX 7: EQUALITY IMPACT ASSESSMENT TOOL 21

1 KEY POINTS OF THIS POLICY:



1 INTRODUCTION

1.1 The purpose of this policy is to set out the selection criteria, training requirements and assessment framework for non-medical authorisation of blood components across the Royal Devon University Healthcare NHS Foundation Trust (RDUH).

1.2 **Failure to comply with this policy could result in disciplinary action.**

2 PURPOSE

2.1 The purpose of this document is to detail the process for a senior Registered Practitioner, working in a relevant specialist role, who wishes to undertake the authorisation of blood component transfusions.

2.2 The policy applies to all non-medical staff.

2.3 Implementation of this policy will ensure that:
Any Registered Practitioner wishing to develop their role to include authorisation of blood component transfusion will have completed the relevant training and competency assessment.

2.4 The Registered Practitioner who takes on the role of blood component authorisation will need continued support and mentorship with a period of supervision prior to completion of competency assessment to allow independent practice.
The authorisation of a blood transfusion must only occur when RDUH policies and guidelines are followed and when there are no alternatives to transfusion.

2.5 Non-medical authorisation is not suitable for all registered practitioners and should only be taken on within agreed governance structures of the Trust after consideration of service and clinical need and at the recommendation of the designated consultant supervisor and assessor.

3 DEFINITIONS

Blood components consist of red cells, platelets, fresh frozen plasma and cryoprecipitate.

4 DUTIES AND RESPONSIBILITIES OF STAFF

There must be an identified service need within the specific department to implement the non-medical authorisation of blood components.

4.1 <u>Senior Matron</u>

- Review the need for this extended practice to be carried out within their areas of responsibility
- Consider individual applications

4.2 Line Manager

- Be aware of and support those who are authorised to work under this extended practice within their area of responsibility
- Ensure that all staff identified within their area of responsibility to work under this policy meet the minimum requirements and remain up to date with all necessary training and assessment requirements identified in this policy

4.3 Consultant Supervisor and Mentor

- They must be a consultant practising in the specialist area in which the practitioner is employed
- They must be up to date with all mandatory training as per RDUH NHS Foundation Trust mandatory training requirements
- They should regularly make decisions to transfuse and authorise blood transfusions and be available for the practitioner to work alongside for learning and assessment purposes
- They are responsible for assuring the competence of the practitioner by providing them with supervision, support and opportunities to develop competence in authorisation of blood components during work-based learning
- They are responsible for supervising the individual during the first 10 non-medical authorisations and provide clinical oversight and governance ongoing.
- They should complete and sign the 'Declaration of competence form' following the completion by the individual of training, assessment and supervision of 10 non-medical authorisations.

4.4 Registered Practitioners working under this Policy

- The registered practitioner must ensure that they have completed and met all of the training and competency requirements for this role
- Identify the need for the competency in order to benefit their patient caseload and agree this with their Senior matron / Lead Nurse
- Identifying and agreeing a suitable Consultant Supervisor and Mentor
- Maintaining accurate documented evidence of training and practice
- They must be up to date with all mandatory training as per RDUH NHS Foundation Trust mandatory training requirements
- Ensure that they are familiar with current local and national guidelines and policies relevant to this sphere of practice
- Is responsible for obtaining adequate knowledge and experience in authorising blood components including a period of supervision prior to the assessment of their competency
- Must keep their training and skills up to date throughout their working life as a non-medical authoriser and have a duty to practice within their own area of competence
- May only authorise blood components within their own specific area or for an identified group of patients.
- Must work within the scope of practice and the relevant council code of contact (NMC or HCPC)
- Must complete their 2 yearly transfusion training as per RDUH NHS Foundation Trust mandatory training requirements
- Must audit their transfusion practice with their supervisor in order to be able to confirm continuing competency at their annual review. These audits should be sent to

the Hospital Transfusion Team (Eastern - rduh.htt@nhs.net) and the blood transfusion specialist nurses (Northern - rduh.ivnurses@nhs.net)
- Strictly adhere to this policy at all times and report any incidents or near misses relating to this policy via the Trust incident reporting system

4.5 Hospital Transfusion Team

- Reviewing and approving the policy in line with National guidance
- Provide training and support where appropriate
- Maintain a register of non-medical authorisers with their training dates
- Approve update to access rights in EPIC in order to be able to authorise blood components

5 PROCESS

5.1 Selection Criteria

- The candidate must be a current, registered practitioner meeting the professional standards of their relevant governing body
- Provide evidence to satisfy senior matron / lead nurse of an appropriate level of knowledge, skills and expertise in a relevant clinical specialty and manage a caseload of patients, or work as part of a clinical team optimising the care of patients who may require a transfusion
- Have an appropriate level of clinical assessment and decision-making skills
- Candidate needs to be approved by senior matron / lead nurse in their area of work

5.2 Training

- The practitioner must attend and complete an approved training programme such as the NHS Blood and Transplant Non-Medical Authorisation of Blood Components course
- The practitioner must submit a portfolio of evidence to demonstrate knowledge and competence to the mentor
- The period of supervision will be at the discretion of the mentor depending on the needs of the individual although it is recommended to be a minimum of 3 months
- On completion of both the training and the (minimum of 10) supervised competency assessments, a competency declaration form must be completed and sent to the Hospital Transfusion Team inbox (Eastern - rduh.htt@nhs.net), the blood transfusion specialist nurses (Northern - rduh.ivnurses@nhs.net), the line manager and Senior Matron (Eastern) and the Service Manager and Lead Clinician for transfusion (Northern).
- The practitioner should complete an audit of their transfusion authorisation annually with their supervisor

6 ARCHIVING ARRANGEMENTS

The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub and QPulse. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely.

7. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF POLICY

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Ensure that patient safety and clinical effectiveness is not adversely affected by the introduction of extended practice	Monitoring of patient safety incidents and near miss events Clinical audit	Hospital Transfusion Team and Patient Blood Management group Practitioner and Supervisor
Practitioner has a duty to ensure that they keep themselves up to date with the policies and procedures associated with transfusion and maintain their competencies to allow them to authorise blood components	Clinical audit Annual Review	Practitioner and Supervisor Practitioner and Senior Matron / Line Manager
The practitioner must complete their 2 yearly transfusion training as per RDUH NHS Foundation Trust mandatory training requirements	Learn+ Records	Learn+ Records
The practitioner is required to audit their transfusion practice yearly and submit this to the Hospital Transfusion Team	Audit documentation	Hospital Transfusion Team rduh.htt@nhs.net Eastern rduh.ivnurses@nhs.net Northern
All incidents will be fully investigated and retraining initiated where appropriate as detailed in the policy. Non-medical authorisers may need to be competency assessed again following an incident and cease to authorise until this has been completed. The Hospital Transfusion Team will inform them and their mentor of the need for this following an incident.	Datix report Investigation reports	Relevant governance groups Hospital Transfusion Team rduh.htt@nhs.net Eastern rduh.ivnurses@nhs.net Northern

8 REFERENCES

None

9 BIBLIOGRAPHY

Department of Health (2005) Blood Safety and Quality Regulations – section 25 The Blood Safety and Quality Regulations 2005 (legislation.gov.uk)

Cottrell S and Davidson A et Al (2022) Clinical Decision-Making and Authorising Blood Component Transfusion – A Framework to Support Non-Medical Healthcare Professionals. Clinical Decision-Making and Authorising Blood Component Transfusion.pdf

Nursing and Midwifery Council (2018) The Code: Professional Standards of Practice and Behaviour for Nurses, Midwives and Nursing Associates. The Code (nmc.org.uk)

Royal Devon University Healthcare NHS Foundation Trust

APPENDIX 1

Application form for Non-Medical Authorisation of Blood Component Transfusion At Royal Devon University Healthcare NHS Foundation Trust

(*Please send via email to the Hospital Transfusion Team at rduh-tr.HTT@nhs.net (East) or rduh.ivnurses@nhs.net (North)*)

Date Form Completed.....

Applicant Name: **Email:**	
Job Title and Band	
Ward / Dept:	**Division:**
Name of Medical Consultant / Supervisor: **Speciality:**	**Email:**
Name of Senior Matron / Lead Nurse:	**Email:**
Summarise your reasons for applying to become a non-medical authoriser of blood components, justifying your rationale for why this qualification is required for your role (please include impact upon patient care): **State the department and patient group you will be undertaking non-medical authorising (NMA) of transfusions for:** **Which blood components will you be authorising? Include the planned frequency of undertaking this work (estimated):**	
APPROVED ☐	**APPROVED** **Chair's approval subject to the following minor amendments being made** ☐
NOT APPROVED **Amendments required by the author – Chair to be sent amended document.** **Approval will be given when the changes are made** ☐	**NOT APPROVED** **Amendments or rewrite required by the author before resubmission to the next committee meeting** ☐
PBMG Chair Name:	**Signature:**

APPENDIX 2

PORTFOLIO OF EVIDENCE to Support Registered Practitioners in Making the Clinical Decision and Providing the Written Authorisation for Blood Component Transfusion.

Name (Candidate):	Name (Consultant Mentor):
Job Title:	Job Title:
Ward / Dept:	Ward / Dept:
Hospital:	Hospital:

Evidence may take the form of:

- Training Records
- Evidence of previous study covering the knowledge requirements in this framework
- Examples of clinical case reports and reflective practice

Understanding of	Knowledge and Competencies	Evidence Submitted

Patient Assessment and Clinical Decision Making • How to take a patient history • Accounting for co-morbidity • Prescribing concomitant drugs • Consent issues	• Understand the requirement to accurately document all actions and conversations with the patient • Ability to take an appropriate medical history • Ability to link the clinical picture with the interpretation of blood results • Can assess the patient is fit for transfusion • Know which concomitant drugs may be required and request them to be additionally prescribed by a doctor • Understand how to explain the risks and benefits of transfusion and available alternatives • Know how and when to provide patient information leaflets • Considers long-term consent requirements when appropriate • Ability to make appropriate referral if the patient refuses blood transfusion	
Interpreting blood results	• Has knowledge of the ABO and RhD blood group systems • Understands the significance of antibodies and reasons for subsequent potential delay in blood provision Can demonstrate knowledge and understanding: • Of normal, variations of normal and abnormal haemoglobin and platelet counts and clotting results • Ability to interpret results and initiate treatment • Know if more tests and / or further evaluation may be required	
Constituents of Blood	Gain insight into structure, function and the production from whole blood of: • Red cells • White cells • Platelets • Plasma Has knowledge of: • Storage • Safe Handling • Temperature control / cold chain requirements	

Indications for the use of specific blood components	• Know the alternatives to consider, if appropriate • Demonstrates a clear understanding of the use of individual blood components • Can define the indications for the use of blood components within their own areas • Uses local policy and national indication codes to demonstrate appropriate use • Can demonstrate in which conditions their use may not be appropriate • Knows when to contact medical / haematology consultant with responsibility for the patient	
Understanding of Anaemia	• Can define the types of anaemia • Understands the physiological process for iron deficiency anaemia • Knows when to refer patients for further investigation and treatment and how to order these • Understands the different types of alternatives available	
Special Transfusion Requirements	Demonstrates knowledge and understanding of: • Which patient groups will have special blood requirements and why • Understands the process of notifying the lab and clinical staff to prevent these patients receiving the wrong blood • Understands the relevance of antibody screening and detection when requesting emergency and routine blood components	
Writing the instruction to transfuse a blood component	Demonstrates knowledge that the written instruction includes: • Patient's full name • Date of birth • Hospital number • Component description • Any special requirements • The length of time the transfusion is to run over • The number of units / volumes of transfusion • The route of administration • Concomitant drugs that need to be administered	

Requesting Blood Components	Has knowledge of local guidelines for requesting blood components and the need to provide the following details: • Full name of the patient • Date of birth • Hospital number • Patient sex • When the patient is to be transfused • How many units and any special requirements • Where the patient is to be transfused • Diagnosis • Indication for transfusion • Relevant transfusion history • Contact name and telephone number	
Pre-transfusion testing process	Can demonstrate an understanding of the: • Pre-transfusion sampling process • Sample labelling requirements • BSH guidelines for pre-transfusion testing • Time limits surrounding the validity of samples in storage • The laboratory processes and timelines for pre-transfusion testing, observing this wherever possible	
Risks and adverse events associated with transfusion	• Demonstrates knowledge and understands the risks of transfusion and can describe the following: • Transfusion and transmitted bacterial and viral infections • Transfusion Related Lung Injury (TRALI) • Acute haemolytic transfusion reaction • Wrong blood to wrong patient • Fluid overload – specific knowledge and awareness around Transfusion Associated Circulatory Overload (TACO) • Anaphylaxis • Iron overload • Can recognise the signs and symptoms of a transfusion reaction • Can describe actions required in an emergency situation	
Legal Responsibilities	• Has knowledge and understanding of the NMC Professional Standards of Practice and behaviour for nurses, midwives and nursing associates or	

	HCPC Standards of conduct, performance and ethics. Record Keeping • Can explain why consent and the reason for transfusion must be recorded in the patient's notes • Can explain why all actions must be documented	
Transfusion guidelines and protocols	Can demonstrate knowledge and understanding of: • SABRE reporting to SHOT / MHRA • Annual SHOT report and recommendations • BSH guidelines • NBTC guidelines • NICE guidelines • Blood Safety and Quality Regulations 2005 • NHSBT and Trust specific patient information leaflets	
Human Factors	• Understands the influence of Human Factors on the safe provision of blood components • Watch Health Education England / Human Factors video (Gina's Story) and provide a reflection / statement of their knowledge regarding Human Factors in the authorisation process for blood components.	

APPENDIX 3

LOG OF SUPERVISED AUTHORISATION OF BLOOD COMPONENTS

Record of supervised authorisation of blood components					
Date	**Patient Hospital Number**	**Component**	**Rationale for transfusion**	**Request for transfusion completed appropriately**	**Mentor approval**

Log of at least 10 supervised authorisations of blood components

Log of transfusions – photocopy as required

Hospital Number	Location	Number of units	Diagnosis
Rationale for the transfusion		Any other medication prescribed	Comments
Signature of Trainee		Date	Time
Signature of mentor / supervisor		Date	Time

Hospital Number	Location	Number of units	Diagnosis
Rationale for the transfusion		Any other medication prescribed	Comments

APPENDIX 4

DECLARATION OF COMPETENCE FORM
Non-medical Authorisation and Written Instruction for Red Cell and Platelet Transfusions

I have met the knowledge and competency criteria and am proficient to undertake the authorisation and written instructions for red cells and platelets.
Name: ……
Signature: …….
Clinical Area / Speciality: ……
Date: ……..

Medical Lead:
I have assessed the above practitioner and deem them proficient to undertake the authorisation and written instructions for red cells and platelets
Name: ……
Signature: …….
Date: …….

Please send a copy of this form to:

- Your Senior Matron / Lead Nurse
- The Specialist Transfusion Practitioner
- The Trust Transfusion Committee
- Keep the original for your own records

APPENDIX 5

NON-MEDICAL AUTHORISING AUDIT TOOL

Name of Authoriser: …….
Auditor / Supervisor: ……
(A minimum of 2 audited transfusions per year required)

Date of transfusion		Time of transfusion (if after 8pm, was overnight transfusion appropriate)	
Ward			
Reason for transfusion			
Pre-transfusion Hb g/l		Post transfusion Hb g/l	
Did the patient have any special requirements?			
Was TACO risk assessed? Which risk factors were present?			
Was a single unit prescribed for non-bleeding adult patient and was patient re-assessed prior to further transfusion?			
Was consent for transfusion documented?			
Were concomitant drugs considered?			

APPENDIX 6: COMMUNICATION PLAN

COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the strategy/policy	Practitioners wishing to undertake this extended role as part of their practice Senior matrons and Lead Nurses Agreed Clinical Mentors Hospital Transfusion Team
The key changes if a revised policy/strategy	Aligned the Eastern Standard Operating Procedure with the Northern Policy
The key objectives	The purpose of this policy is to set out the selection criteria, training requirements and assessment framework for non-medical authorisation of blood components across the Royal Devon University Healthcare NHS Foundation Trust (RDUH)
How new staff will be made aware of the policy and manager action	If a need is identified for a non-medical authoriser of blood components for transfusion, line managers or transfusion practitioner should make potential candidates aware of the policy.
Specific Issues to be raised with staff	Non-medical authorisers of blood components for transfusion must fully comply with this policy Staff already performing non-medical authorisation of blood components will be made aware that this policy has been updated and aligned across sites
Training available to staff	No specific training required for this policy
Any other requirements	None identified
Issues following Equality Impact Assessment (if any)	No negative impacts
Location of hard / electronic copy of the document etc.	This policy will be stored on the laboratory Q-Pulse document control system and accessible on the Trust Intranet

APPENDIX 7: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	**Non-Medical Authorisation of Blood Components for Transfusion by Registered Practitioners Policy**
Division/Directorate and service area	Specialist Services / Transfusion
Name, job title and contact details of person completing the assessment	Katy Cowan, Transfusion Practitioner Extn 2932 kate.cowan@nhs.net
Date completed:	30/06/2023

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

The purpose of this policy is to set out the selection criteria, training requirements and assessment framework for non-medical authorisation of blood components across the Royal Devon University Healthcare NHS Foundation Trust (RDUH).

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☐ Staff ☒ Patients ☒ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☐	☒
Disability	☐	☒
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒
Race	☐	☒
Religion / belief	☐	☒
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒